

Participant information sheet and consent form

Template in plain language

Foundations of Clinical Research · Template version 1.0

Writing for participants: readability guidance (delete this box)

- Write as you would speak to a patient or relative: short sentences (under 20 words), everyday words (“drip” not “intravenous infusion”), and “you” rather than “the participant”. Explain any medical word the first time you use it.
- Aim for a reading age of about 12 years. In Word: File → Options → Proofing → Show readability statistics; aim for a Flesch Reading Ease of 60 or more.
- Use the questions below as headings. Keep the information sheet to 3–5 pages, font at least 11 pt.
- Translate into the languages your patients speak, have it back-translated, and test it with two or three lay people before you submit it.
- Replace the Neudata header with your institution’s letterhead, and put the version number and date in the footer of every page. The ethics committee approves a specific version.
- Text in normal black is suggested wording you may adapt; [grey brackets] must be completed.

Study title (plain): *[Short title in everyday words, e.g. “How much drip fluid is best for people with severe infection?”]*

Full title: *[Full scientific title]*

Study number / version: *[Protocol number]* · Information sheet version *[1.0]* of *[DD/MM/YYYY]*

Principal investigator: *[Name], [Department], [Institution]*

Part A — Participant information sheet

1 Invitation

We invite you to take part in a research study. Before you decide, it is important that you understand why the research is being done and what it will involve. Please take time to read this information, and talk to others if you wish. Ask us if anything is not clear.

2 Why is this study being done?

[Explain the problem and the question in 2–3 short sentences.]

Example wording: *Doctors do not yet know how much drip fluid is best for people with a severe infection called septic shock. Some think less fluid may be better. This study will find out whether giving less fluid is safe and helpful.*

3 Why have I been asked?

You have been asked because *[reason, e.g. you are being treated in the intensive care unit for a severe infection]*. About *[number]* people will take part.

4 Do I have to take part?

No. It is up to you. If you decide to take part, you will be asked to sign a consent form and you will keep a copy of this sheet. You are free to stop at any time, without giving a reason. Deciding not to take part, or stopping, will not change the care you receive in any way.

5 What will happen if I take part?

[Describe, step by step and in time order, what will happen: visits, tests, extra blood samples (how much, in teaspoons), questionnaires, how long each takes, and for how long you will be in the study. Say clearly what is research and what is normal care.]

Example wording: *We will collect information from your medical records every day for the first 5 days in the ICU. No extra tests are needed. After 28 days we will check your health from your records, or by a short phone call.*

6 What are the possible risks or discomforts?

[List the risks honestly, how likely they are, and what will be done to reduce them. If there are no extra physical risks, say so, and mention the small risk to privacy.]

7 What are the possible benefits?

[Be honest. Often: "There may be no direct benefit to you, but the results may help patients like you in the future."]

8 What if something goes wrong, or new information becomes available?

If you are harmed by taking part, *[describe insurance / compensation arrangements as required by National regulations]*. If we learn anything new that might change your wish to stay in the study, we will tell you.

9 Will my information be kept confidential?

Yes. Your information will be given a study number and your name will not appear on any study form. The list that links your name to your study number is kept locked by the study doctor. Only the study team, *[Ethics committee name]* and authorised inspectors may look at your records, to check the study is done properly. Your information will be kept for *[number]* years, as required by *[National regulations]*, and then destroyed safely.

10 What will happen to the results?

The results will be shared with doctors and nurses, and published in a medical journal. You will not be identified in any report. If you would like a summary of the results, tell the study team.

11 Will I be paid, or will it cost me anything?

[Explain any costs (none should fall on the participant) and any reimbursement of travel or time. Payments must not be so large that they pressure people to take part.]

12 Who has reviewed this study?

This study has been reviewed and approved by *[Ethics committee name]* (reference *[number]*).

13 Who can I contact?

[Give the name, phone number and email of a study team member who can answer questions, AND an independent contact (e.g. the ethics committee office) for complaints or questions about your rights.]

Research in emergencies and intensive care

When the patient cannot decide for themselves (guidance — keep only what applies)

- **Consent from a representative.** If the patient lacks capacity (sedation, shock, delirium), a legally authorised representative (often the next of kin) may give consent on their behalf, if [National regulations] allow it. The representative should decide on the basis of what the patient would have wanted. Prepare a version of this sheet addressed to the representative (“your relative”).
- **Deferred consent.** When treatment must start within minutes or hours and no representative is available, some regulations allow the patient to be included first and consent to be asked as soon as possible afterwards. This must be approved in advance by the ethics committee.
- **When the patient recovers capacity,** they must be informed and asked for their own consent to continue (see the continuing-consent section in Part B). Explain what happens to data already collected if they refuse, and what happens if the patient dies before consent could be obtained.
- **Waiver of consent** may be granted by the committee for minimal-risk studies of routine records when asking consent is impracticable. A short information notice and an opt-out are still good practice.

Part B — Consent form

Study title: [Study title]
 Participant study ID: []-[]-[]-[]-[]-[]
 Principal investigator: [Name]

The participant writes their initials in each box; ticks are not enough. Optional statements have Yes / No boxes and must not affect participation.

#	Statement	Initials
1	I confirm that I have read (or had read to me) the information sheet version [1.0] dated [DD/MM/YYYY] for this study. I have had the chance to think about it and to ask questions, and my questions have been answered.	
2	I understand that taking part is voluntary and that I am free to stop at any time, without giving a reason, and without my medical care or legal rights being affected.	
3	I understand that my medical records and study data may be looked at by authorised people from the study team, [Ethics committee name] and regulatory authorities, and I give permission for this.	
4	I understand that if I stop taking part, [the information already collected will / will not be used].	
5	OPTIONAL: I agree that my anonymised data may be used in future research approved by an ethics committee. ☐ Yes ☐ No	
6	I agree to take part in this study.	

Participant

Name Signature Date (DD/MM/YYYY) and time

Person taking consent

A member of the study team trained in the protocol and in Good Clinical Practice, and delegated this task by the principal investigator.

Name and role Signature Date (DD/MM/YYYY) and time

Impartial witness (if the participant cannot read or write)

I confirm that the information was read accurately to the participant, who had the chance to ask questions, and that consent was given freely. The participant may make a thumbprint instead of a signature.

Witness name Signature Date (DD/MM/YYYY)

Legally authorised representative (if the patient cannot consent)

I am giving consent on behalf of the patient named above, who is not able to decide for themselves at present. I believe that taking part is in line with what they would want.

Name and relationship to patient

Signature

Date (DD/MM/YYYY) and time

Continuing consent after a period without capacity (deferred consent)

To be completed by the participant as soon as they are able to decide, if they were included with representative or deferred consent.

I have been told that I was included in this study while I was too unwell to decide.

☐ I agree to continue in the study.

☐ I do not wish to continue, but the information collected so far may be used.

☐ I do not wish to continue and I want my information removed, where this is still possible.

Participant (continuing consent)

Name

Signature

Date (DD/MM/YYYY) and time

Copies: 1 for the participant (with the information sheet), 1 for the study file, 1 for the medical record (if required by [National regulations]).